

Worked Synthetic Patient Review Cases

Gold-standard formatting examples for retrieval and evaluation

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Document control and RAG metadata

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Authority	Clinical Education Lead
Supersedes	None - initial synthetic edition
Applicability	Training simulations at Northbridge Cardiology Practice only

Retrieval rule

Retrieve only for formatting and evaluation. Never treat a worked case as evidence for another patient or copy its dose into another record.

1. Use of worked cases

These examples are fictional outputs demonstrating completeness. They are not clinical guidance. Each medication instruction is tied to a fictional signed order in the case. Similarity to another patient does not authorize reuse.

2. Case A - incomplete authorization

Synthetic facts: Patient TRN-A01 has an active medicine name but no dose unit, a recent laboratory result flagged outside the supplied range, and no named result reviewer.

Gold-standard response

- Medication recommendation: No final change recommendation can be issued. Status: AWAITING CLARIFICATION.
- Warnings: incomplete order and unreviewed flagged result create a safety stop.
- Further tests: none invented; route existing result to duty cardiology clinician today and ask clinician whether repeat testing is required.
- Instructions: withheld because dose/unit is incomplete.
- Appointment: same-day medication reconciliation/clinical triage; booking owner medication review desk; results owner duty clinician.
- Citations: NB-GOV-101 section 2; NB-SAF-401 section 3; NB-DIAG-501 section 3.

3. Case B - authorized continuation with monitoring

Synthetic facts: Patient TRN-B02 has a complete signed continuation order for Fictionalmed 10 mg tablets: take one tablet by mouth once every morning from 18 August 2026 until cardiology review. The signed plan orders renal function and electrolytes for 25 August 2026, results owner Dr Tariq Bell, and a nurse review within 2-4 weeks.

Gold-standard response

- Medication recommendation: Continue Fictionalmed exactly as authorized; do not adjust independently.
- Exact instructions: Take one 10 mg tablet by mouth once every morning, starting 18 August 2026, until reviewed by cardiology.

- Warnings: seek same-day review for severe dizziness, fainting, or rapidly worsening symptoms; use emergency pathway for severe symptoms.
- Further tests: renal function and electrolytes on 25 August 2026; diagnostics booking owns scheduling; Dr Tariq Bell owns result review.
- Appointment: nurse monitoring review between 1 and 15 September 2026; bring/currently confirm medication list and report symptoms.
- Missing information: medicine-specific food and missed-dose wording is absent; pharmacist must supply it from authorized product information.
- Citations: signed fictional order; NB-MED-201 sections 1 and 3; NB-OPS-301 section 5; NB-SAF-401 section 5.

4. Case C - anticoagulant procedure conflict

Synthetic facts: Patient TRN-C03 has a current signed anticoagulant order and reports a dental procedure tomorrow, but there is no peri-procedure plan.

Gold-standard response

- Medication recommendation: Continue the current signed order unless and until the responsible clinician issues a procedure-specific instruction; urgent clarification is required today.
- Warnings: do not independently skip, double, or alter doses; significant bleeding or head injury follows the urgent/emergency pathway.
- Further tests: do not invent testing; provide current renal function and complete medication list to the reviewing clinician.
- Appointment/action: same-day anticoagulation/prescriber review; verified communication with dental team through authorized staff.
- Citations: NB-SAF-402 section 2; NB-MED-202 section 1; NB-OPS-301 section 2.

5. Case D - retrieval failure

Synthetic facts: Patient TRN-D04 is verified, but the guideline retriever is unavailable and the requested recommendation depends on current guideline evidence.

- Return verified patient facts and existing signed orders only.
- State that the evidence retrieval service failed and no evidence-backed new recommendation can be completed.
- Route to clinician/pharmacist review and offer retry; do not substitute model memory.
- Record tool failure without identifiable content.
- Citations: NB-RAG-000 section 5; NB-SAF-401 section 3.

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